

— AN NP STUDENT'S GUIDE —

The Peptide *Primer*

A 2026 guide to understanding a moving landscape

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quick reference

The Peptide Status Grid

as of July 13, 2026 — before PCAC review July 23-24

Peptide	Studied for	FDA status	Evidence	Page
Semaglutide	Weight loss, T2 diabetes	FDA-Approved	<i>Strong</i>	10
Tirzepatide	Weight loss, T2 diabetes	FDA-Approved	<i>Strong</i>	11
PT-141 (Vyleesi)	Sexual function	FDA-Approved	<i>Moderate</i>	18
Teriparatide	Osteoporosis	FDA-Approved	<i>Strong</i>	19
BPC-157	Tissue repair, gut	Limbo — PCAC 7/23	<i>Mostly animal</i>	13
TB-500	Recovery, healing	Limbo — PCAC 7/23	<i>Thin</i>	14
CJC-1295	Anti-aging, muscle	Limbo — PCAC declined	<i>Limited</i>	15
Ipamorelin	Anti-aging, muscle	Limbo — PCAC declined	<i>Limited</i>	15
Thymosin Alpha-1	Immune support	Limbo — PCAC declined	<i>Moderate (Int'l)</i>	16
NAD+	Energy, cellular repair	Not classified as peptide	<i>Moderate</i>	17
GHK-Cu	Skin, healing	Limbo — review by 2/27	<i>Moderate</i>	16
Melanotan II	Tanning	Likely to stay restricted	<i>Safety concerns</i>	16

Peptide	Studied for	FDA status	Evidence	Page
AOD-9604	Fat loss	PCAC declined 2024	Weak	16

LEGEND

	FDA-approved — regulated pathway		No legal compounding pathway
	Under active PCAC review		Safety concerns / restricted

quick reference

The Two Questions

that separate a legitimate peptide provider from everything else

01

"Who is the pharmacy filling this prescription, and can I verify their license?"

A legitimate provider works with a state-licensed 503A pharmacy or an FDA-registered 503B outsourcing facility. Both are publicly verifiable on state pharmacy board websites. If the answer is vague — or the peptide arrives without a pharmacy name attached — you are not in the regulated market.

02

"What batch-specific testing was done, and can I see the Certificate of Analysis?"

A Certificate of Analysis (COA) verifies identity, potency, sterility, and endotoxin limits for the specific batch you're receiving. Legitimate pharmacies provide them on request. Generic (non-batch-specific) documents don't count. If no COA is available at all, walk away.

The full explanation lives on page 20.

Before you read

This document is education, not medical advice.

I am a Family Nurse Practitioner student at Emory University. I am not yet a licensed prescriber. Nothing in this document is a recommendation, prescription, or clinical directive. I am summarizing published research so you can have better conversations with a licensed clinician — not so you can self-treat.

Peptides are pharmacologically active substances. Some are FDA-approved drugs. Some exist in regulatory limbo. Some are prohibited from legal compounding. All of them, taken without clinical supervision, carry real risks.

If you are considering peptide therapy, work with a licensed clinician who can review your history, order appropriate labs, and monitor you. Do not order peptides from research-chemical websites for personal use. That is where the actual danger lives.

About the moving landscape

The peptide regulatory environment changed dramatically between February and April of 2026, and it is still changing. What was prohibited in January may be under review in July. What is compoundable this month may not be next month.

This document reflects my understanding of the landscape as of July 13, 2026, before the FDA's Pharmacy Compounding Advisory Committee (PCAC) meeting scheduled for July 23-24, 2026. I plan to revise this document after that meeting.

How to use this document

The cheat sheet at the front is meant to be genuinely useful in 30 seconds. Sections 1 and 2 give you the regulatory framework you need to evaluate any peptide. Sections 3, 4, and 5 are peptide-by-peptide cards you can read individually — designed to be screenshot-friendly and shareable.

Every clinical claim in this document has a source. Where the evidence is strong, I say so. Where it's weak, I say that too.

A note about my disclosures

Pamela Mahan, PA, is the founder of Youthful Medical Spa in Ponte Vedra, Florida. Where I currently work while still in school and earn a commission on patients

who book with the practice through content I create. Some of the peptides discussed in this document are offered at Youthful. This affiliation shapes my exposure to the space; I have tried to keep the content in this document as evidence-forward as possible regardless.

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1. What peptides actually are

Peptides are short chains of amino acids. Amino acids are the building blocks of proteins. A peptide is what you get when a small number of amino acids (typically fewer than 50) are linked together in sequence. Anything much longer than that is called a protein. This is a somewhat arbitrary distinction, but it matters for how these molecules are regulated: peptides and proteins fall under different FDA pathways, and manufacturing them is chemically different.

Your body makes thousands of peptides constantly. Insulin is a peptide. Oxytocin is a peptide. Every hormone that involves signaling between cells is either a peptide or a small protein. When people talk about "peptide therapy," they usually mean administering a specific peptide from outside the body — either one your body already makes, one that mimics something your body makes, or a novel peptide developed for a specific effect.

How peptides work in the body

Most peptides work by binding to receptors on the surface of cells and triggering a signaling cascade. This is fundamentally different from how, say, a small molecule drug like ibuprofen works. Peptides tend to be highly specific — they interact with one or a small number of receptors — which is part of why the wellness world got excited about them. In theory, this means fewer off-target side effects than a traditional drug.

In practice, "in theory" is doing a lot of work in that sentence. Specificity is one variable. What matters clinically is the whole picture: absorption, whether the peptide survives digestion (most don't, which is why almost all therapeutic peptides are injected), how quickly it's cleared from the body, what happens when it binds to its receptor, and whether it triggers an immune response.

Why peptides became a wellness trend

Two things happened in the mid-2020s. First, semaglutide (Ozempic, Wegovy) and later tirzepatide (Mounjaro, Zepbound) demonstrated in large randomized trials that GLP-1 receptor agonist peptides could produce sustained weight loss in ways nothing before them had. This shifted mainstream medicine's view of peptides as a therapeutic category.

Second, biohacker and functional medicine communities began popularizing a wider range of peptides — some with genuine research behind them, some with almost none — as tools for recovery, longevity, cognitive performance, and

aesthetics. This created enormous consumer demand. Compounding pharmacies filled that demand. Telehealth clinics scaled it. And the "research chemical" market — technically-not-for-human-use vials shipped from overseas — exploded to fill the gaps the regulated market couldn't.

2. The regulatory framework

When you read about a peptide online, the most important question is not "does it work?" but "what is its legal and regulatory status?" That status determines whether you can legally get it, whether the source is subject to safety testing, and whether a licensed clinician can legally prescribe it.

There are four categories a peptide can fall into.

Category 1 — FDA-approved drugs

The highest bar. An FDA-approved peptide drug has gone through preclinical testing, then Phase 1, 2, and 3 human trials, and been reviewed by the FDA. It has a New Drug Application (NDA) on file, established indications, standardized dosing, known safety profile, and monitoring requirements.

Examples: semaglutide (Ozempic, Wegovy, Rybelsus), tirzepatide (Mounjaro, Zepbound), liraglutide (Victoza, Saxenda), dulaglutide (Trulicity), PT-141 as bremelanotide (Vyleesi), and older peptides like insulin, oxytocin, teriparatide, and octreotide.

Category 2 — Compounded from FDA-approved sources

A licensed compounding pharmacy can prepare a drug for an individual patient with a specific prescription. This is legal under Section 503A. Larger-scale compounding without patient-specific prescriptions falls under Section 503B. In April 2026, the FDA proposed excluding semaglutide, tirzepatide, and liraglutide from the 503B bulks list, closing the large-scale compounding pathway for GLP-1s. Patient-specific 503A compounding still exists but is constrained by the "essentially a copy" rule.

Category 3 — Compounded from bulk substances on the 503A Bulks List

For substances that are not FDA-approved drugs but have a documented history of clinical use, the FDA maintains a "503A Bulk Drug Substances List." Substances on this list can be legally compounded. Substances the FDA has flagged as "Category 2" — safety concerns — are effectively prohibited from compounding.

This is the category where most of the drama lives. In September 2023, the FDA moved over a dozen popular peptides into Category 2. In February 2026, HHS Secretary RFK Jr. announced intent to reclassify around 14 back toward Category

1. On April 22, 2026, the FDA formally removed 12 peptides from Category 2 — but did not simultaneously add them to Category 1.

What this means right now (July 2026):

Twelve peptides — BPC-157, TB-500, KPV, MOTS-C, DSIP, Semax, Epitalon, LL-37, Dihexa, GHK-Cu, PEG-MGF, and Melanotan II — are in regulatory limbo. No longer flagged as safety concerns, but not authorized for compounding either.

Seven of these will be formally reviewed by the FDA's Pharmacy Compounding Advisory Committee (PCAC) on July 23-24, 2026. The remaining five will be reviewed before February 2027.

PCAC's vote is advisory. Formal FDA rulemaking is still required — a process that typically takes over a year.

This is why "peptides are legal again" headlines are misleading. The reality is more complicated.

Category 4 — "Research chemicals" and gray-market sources

This is the category most consumers accidentally end up in. Websites sell peptides labeled "for research use only, not for human consumption." This labeling is a legal fiction — the manufacturer knows the vials are being purchased for personal injection. But the labeling means these products are not manufactured to pharmaceutical standards. They may lack stability, potency, endotoxin, and sterility validation. Some are made in facilities with no meaningful quality control.

Contaminated peptide preparations have been linked to injection site infections, systemic inflammatory responses, and unpredictable dose responses. No licensed clinician should be sourcing patient care through these channels.

— SECTION THREE —

GLP-1s

The category with the deepest research base and the clearest regulatory status

Semaglutide

O Z E M P I C • W E G O V Y • R Y B E L S U S

STUDIED IN

Type 2 diabetes, obesity, cardiovascular risk reduction, sleep apnea, knee osteoarthritis.

MECHANISM

- Activates the GLP-1 receptor, mimicking a gut hormone released after eating
- Slows gastric emptying and signals satiety to the brain
- Enhances glucose-dependent insulin release from the pancreas
- Longer half-life than natural GLP-1 (once-weekly dosing)

EVIDENCE LEVEL

Strong. Multiple large randomized trials, including STEP-1 (~14.9% average weight loss over 68 weeks) and SELECT (cardiovascular outcome reduction in people with obesity and established cardiovascular disease).

FDA STATUS

FDA-Approved

NOTES

Established side effects include gastrointestinal symptoms during titration, a black-box warning for thyroid C-cell tumors based on rodent studies, pancreatitis risk, and a growing signal for gastroparesis in some patients. Muscle mass loss with rapid weight loss is a real concern and part of why clinical monitoring matters. Real-world weight loss tends to run below trial averages (8-12% at 12 months in remote programs), reflecting adherence, dose escalation, and access differences.

F O R W E I G H T A N D M E T A B O L I C H E A L T H

Tirzepatide

M O U N J A R O • Z E P B O U N D

STUDIED IN

Type 2 diabetes, obesity, obstructive sleep apnea, cardiovascular outcomes.

MECHANISM

- Dual agonist — activates both GLP-1 and GIP receptors
- GIP (glucose-dependent insulintropic polypeptide) adds an additional pathway for glucose and appetite regulation
- Once-weekly subcutaneous injection

EVIDENCE LEVEL

Strong. SURMOUNT-1 (~20.9% weight loss over 72 weeks at highest dose). SURMOUNT-5, the first head-to-head randomized trial vs semaglutide (2025), showed ~20.2% vs ~13.7% weight loss over 72 weeks.

FDA STATUS

FDA-Approved

NOTES

Side effect profile similar to semaglutide — GI symptoms during titration, same thyroid warning. Generally higher cost than semaglutide. The dual mechanism appears to be responsible for the greater weight loss magnitude on average.

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— SECTION FOUR —

In Limbo

*Peptides in a regulatory gray zone — where marketing has outpaced the human
clinical evidence*

STUDIED FOR TISSUE REPAIR

BPC-157

BODY PROTECTIVE COMPOUND - 157

STUDIED IN

Wound healing, tendon and ligament repair, gastrointestinal protection, inflammation.

MECHANISM

- Synthetic fragment of a protein found in human gastric juice
- Proposed mechanisms include angiogenesis, nitric oxide modulation, and effects on growth factor receptors
- Note: proposed mechanisms in humans are inferred from animal work

EVIDENCE LEVEL

Mostly animal. Over 100 preclinical studies in rats and mice. Human trials are few, small, and typically not blinded or placebo-controlled to modern standards.

FDA STATUS

Under PCAC review 7/23

NOTES

One of the most heavily marketed peptides — and one where the marketing has clearly outpaced the human clinical evidence. Was Category 2 (2023–April 2026), currently in regulatory limbo. FDA safety concerns originally cited immunogenicity risk, peptide-related impurities from compounding, and limited human data. PCAC will review July 23–24, 2026 for potential 503A Bulks List inclusion.

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STUDIED FOR TISSUE REPAIR

TB-500

THYMOSIN B - 4 FRAGMENT

STUDIED IN

Wound healing, tissue repair, muscle recovery, cardiac protection.

MECHANISM

- Synthetic fragment of thymosin beta-4, a naturally occurring peptide
- Thymosin beta-4 is involved in cell migration, actin sequestration, and tissue repair
- Effects in humans at injected doses of the fragment specifically are not well characterized

EVIDENCE LEVEL

Thin. Preclinical work suggests roles in tissue repair and cardiac protection. Robust human trial data is limited.

FDA STATUS

Under PCAC review 7/23

NOTES

Was Category 2, removed April 2026 but not authorized for compounding. Same immunogenicity and impurity concerns cited by FDA as with BPC-157. Under formal PCAC review July 23-24, 2026.

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STUDIED FOR GROWTH HORMONE MODULATION

CJC-1295 + Ipamorelin

COMMONLY PRESCRIBED AS A STACK

STUDIED IN

Growth hormone and IGF-1 elevation, body composition, sleep quality, aging-related concerns.

MECHANISM

- CJC-1295 is a GHRH (growth hormone-releasing hormone) analog that stimulates growth hormone release
- Ipamorelin is a growth hormone secretagogue that works through the ghrelin receptor pathway
- The stack is designed to amplify pulsatile growth hormone release

EVIDENCE LEVEL

Limited. Studies exist documenting GH and IGF-1 elevation. Long-term safety data at wellness-context doses is not well established.

FDA STATUS

PCAC declined 2024

NOTES

Was Category 2, removed April 2026 but currently in limbo. Important nuance: PCAC previously voted against inclusion in 2024. Elevated growth hormone over time carries theoretical risks that are not benign — insulin resistance, cardiac hypertrophy, and increased cancer risk are concerns that require monitoring by a licensed clinician if considered.

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STUDIED FOR IMMUNE MODULATION

Thymosin α -1

THYMALFASIN

STUDIED IN

Chronic viral infections (hepatitis B, hepatitis C), immune modulation, cancer-adjunct therapy.

MECHANISM

- Naturally occurring peptide produced by the thymus
- Modulates T-cell function and other immune signaling
- Well-characterized immunomodulatory activity in vitro and in vivo

EVIDENCE LEVEL

Moderate (international). Approved and used therapeutically in over 30 countries — but not FDA-approved in the US. Human evidence base is more substantial than BPC-157 or TB-500.

FDA STATUS

PCAC declined 2024

NOTES

The most well-established peptide in this section from a research standpoint. Currently unavailable in the US through legal compounding channels. Was Category 2, removed April 2026, but PCAC voted against inclusion in 2024. Regulatory status is genuinely frustrating for patients and clinicians familiar with international use.

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STUDIED FOR CELLULAR ENERGY

NAD+

NICOTINAMIDE ADENINE DINUCLEOTIDE

STUDIED IN

Mitochondrial function, cellular repair pathways, cognitive aging, metabolic health.

MECHANISM

- A coenzyme present in every cell — declines with age
- Central to mitochondrial energy production and sirtuin (longevity gene) activation
- Direct IV infusion vs. precursor supplementation (NMN, NR) are different clinical propositions

EVIDENCE LEVEL

Moderate for precursor supplementation in humans; more limited for direct IV protocols. Substantial preclinical rationale.

FDA STATUS

Not classified as peptide

NOTES

Technically not a peptide — NAD+ is a coenzyme, not a chain of amino acids. Included here because it's widely marketed alongside peptides in the same wellness contexts and often bundled at med spas. As an IV therapy or injection, regulatory considerations differ from compounded peptides. Precursor supplements (NMN, NR) have separate FDA history.

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STUDIED FOR VARIED INDICATIONS

Also in limbo

A QUICK PASS AT OTHER PEPTIDES YOU'LL SEE
IN THIS SPACE

KPV

Anti-inflammatory peptide. Under PCAC review July 2026. Currently in limbo.

MOTS-C

Mitochondrial-derived peptide studied for metabolic health. Under PCAC review July 2026. Currently in limbo.

DSIP / SEMAX / EPITALON

Neurological, nootropic, and anti-aging peptides. All under PCAC review July 2026. All currently in limbo.

GHK-CU

Copper peptide with skin and healing research base. Under PCAC review before February 2027. Currently in limbo.

MELANOTAN II

Tanning peptide. Not expected to move to Category 1 due to safety concerns. Effectively restricted.

AOD-9604

Growth hormone fragment marketed for fat loss. PCAC declined inclusion in 2024. In limbo but unlikely to be authorized.

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— SECTION FIVE —

FDA-Approved

*Peptide medications with regulatory certainty — the safest path if your clinical
need aligns*

STUDIED FOR SEXUAL FUNCTION

PT-141

BREMELANOTIDE • VYLEESI

STUDIED IN

Hypoactive sexual desire disorder (HSDD) in premenopausal women.

MECHANISM

- Melanocortin receptor agonist — acts on the central nervous system
- Different mechanism from vascular-focused erectile dysfunction drugs
- Subcutaneous injection prior to anticipated sexual activity

EVIDENCE LEVEL

Moderate. FDA-approved 2019 based on RECONNECT trials showing modest but statistically significant improvement in sexual desire scores.

FDA STATUS

FDA-Approved

NOTES

Real-world use has been limited by side effects including nausea, flushing, and injection-site reactions. Represents a well-characterized peptide with clear regulatory status — a useful reference point for what "approved peptide medication" looks like.

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S T U D I E D F O R B O N E H E A L T H

Teriparatide

F O R T E O (A L S O : A B A L O P A R A T I D E A S T Y M L O S)

STUDIED IN

Osteoporosis with high fracture risk in men and postmenopausal women.

MECHANISM

- Recombinant fragment of parathyroid hormone (PTH 1-34)
- Stimulates osteoblast activity, increasing bone formation
- Daily subcutaneous injection

EVIDENCE LEVEL

Strong. Multiple randomized trials showing reduced fracture risk in osteoporosis. Well-established clinical use.

FDA STATUS

FDA-Approved

NOTES

Relatively narrow indication (severe osteoporosis) but represents a well-validated peptide therapy category. Course of treatment is typically limited to 2 years due to theoretical concerns about osteosarcoma risk seen in rodent studies.

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Also approved

A P A R T I A L L I S T O F P E P T I D E M E D I C A T I O N S W I T
H F D A A P P R O V A L

OCTREOTIDE (SANDOSTATIN)

Synthetic somatostatin analog. FDA-approved for acromegaly and certain neuroendocrine tumors. Long history of clinical use.

INSULIN

Technically a peptide hormone. In use for over a century — a reminder that "peptide therapy" is not new.

OXYTOCIN (PITOCIN)

Peptide hormone, FDA-approved for labor induction and postpartum hemorrhage prevention.

LIRAGLUTIDE (VICTOZA, SAXENDA)

Older GLP-1 receptor agonist. Daily rather than weekly. Similar indications to semaglutide.

DULAGLUTIDE (TRULICITY)

Another GLP-1 receptor agonist for type 2 diabetes. Weekly dosing.

This is not an exhaustive list. Many older peptide medications exist for narrow indications; the point is that "FDA-approved peptide medication" is a real and stable category that a licensed prescriber can navigate for you.

6. Sourcing: legitimate vs. sketchy

This is the section that will save you the most money and risk. The peptide market is full of well-designed websites selling variously legal, semi-legal, and outright illegal products. Learning to spot the difference is not hard once you know what to look for.

Green flags — signs of a legitimate provider

- Your peptide is prescribed by a licensed clinician (MD, DO, NP, or PA) who has reviewed your history and ordered relevant labs.
- The compounding pharmacy is a state-licensed 503A pharmacy or a registered 503B outsourcing facility. Both are lookupable on state pharmacy board websites and, for 503B, the FDA registered outsourcing facility database.
- Each batch has a Certificate of Analysis (COA) verifying identity, potency, sterility, and endotoxin limits.
- The clinician has a monitoring plan: baseline labs, follow-up labs, clear stopping criteria, and a taper protocol where appropriate.
- The prescriber is willing to say "I don't know" or "there isn't enough evidence for that" when appropriate.

Red flags — signs to walk away

- Product is labeled "for research use only" or "not for human consumption."
- You order the peptide directly from a website with no prescription required.
- The seller is based overseas, ships in unmarked packaging, or requires cryptocurrency payment.
- There is no COA available, or the COA is generic (not batch-specific).
- The clinician (if there is one) has never met with you, does not order labs, and prescribes based on a short online form.
- The provider or influencer promoting the peptide is also selling it directly to you — vertical integration between prescriber and seller creates conflicts of interest.
- Marketing language includes "cures," "reverses aging," "no side effects," "safe for anyone." Real medicine talks in probabilities and tradeoffs.
- The provider cannot or will not tell you which pharmacy is filling your prescription.

The two questions

When you're evaluating any peptide provider or source, ask:

1. "Who is the pharmacy filling this prescription, and can I verify their license?"
2. "What batch-specific testing was done on this preparation, and can I see the Certificate of Analysis?"

If a provider cannot answer both of these clearly, you are not in the regulated peptide market. You are somewhere else.

7. Questions to ask a prescriber

If you're considering peptide therapy under clinical supervision, here are the questions worth asking. A good prescriber will welcome them.

About the peptide itself

- What is the current FDA regulatory status of this peptide, and does that affect how you're prescribing it?
- What does the human clinical evidence actually show for this peptide, at this dose, for this indication?
- What are the known side effects and risks? What are the theoretical risks that don't have long-term human data yet?
- Are there interactions with any medications I'm taking?

About the source

- Which pharmacy will be filling this prescription, and what type of licensure do they have?
- Is a Certificate of Analysis available for the batch I'll be receiving?
- What is the expected shelf life once I receive the product, and how should it be stored?

About monitoring

- What baseline labs will you order before I start?
- What follow-up labs will you order, and how frequently?
- What are the signs that I should stop or adjust the dose?
- What's the plan for tapering off, if applicable?

About the relationship

- If I have side effects, how quickly can I reach you?
- If I want to discontinue, what does that conversation look like?
- What's the total cost — visits, labs, and the peptide itself — over the course of a typical treatment?

8. Further reading

These are the sources I've been reading to build my own understanding. Not all will apply to every reader, but if you want to go deeper, these are honest starting points.

Regulatory

- FDA, Bulk Drug Substances Nominated for Use in Compounding Under Section 503A. [fda.gov](https://www.fda.gov)
- FDA, Pharmacy Compounding Advisory Committee meeting notices and materials. [fda.gov/advisory-committees](https://www.fda.gov/advisory-committees)
- FDA, Interim Policy on Compounding Using Bulk Drug Substances Under Section 503A.

GLP-1 trials

- STEP-1 trial: Wilding JPH et al. Once-Weekly Semaglutide in Adults with Overweight or Obesity. *N Engl J Med.* 2021;384:989.
- SURMOUNT-1 trial: Jastreboff AM et al. Tirzepatide Once Weekly for the Treatment of Obesity. *N Engl J Med.* 2022;387:205.
- SELECT trial: Lincoff AM et al. Semaglutide and Cardiovascular Outcomes in Obesity without Diabetes. *N Engl J Med.* 2023;389:2221.
- SURMOUNT-5 (head-to-head): published 2025.

Peptide regulatory analysis

- Sheppard Mullin, "What to Watch: Status Update on Peptide Regulation."
- Pharmacy Times, "The Peptide Reclassification Everyone's Talking About."
- Lengea Law, "FDA Puts BPC-157, TB-500, and 5 Other Peptides Under the Microscope."

Peptide-specific

- Sikiric P et al. Body Protection Compound-157 (BPC-157) preclinical review series. Extensive animal-model literature.
- King R, Tuthill C. Immune modulation with thymosin alpha 1 treatment. *Vitam Horm.* 2016;102:151-178.

a note on disclosure

About Youthful Medical Spa

Ponte Vedra Beach, Florida

Pamela Mahan, PA, founded Youthful Medical Spa in 2006. She has practiced medicine for over 20 years across family medicine, natural hormone therapy, and aesthetics, and personally trained on the treatments offered at the practice.

I plan to join Youthful as a Family Nurse Practitioner after I graduate from Emory in December 2027. In the meantime, I create content that occasionally references the practice, and I earn a commission on patients who book with Youthful through my content or referrals.

Some of the peptide therapies discussed in this guide are offered at Youthful. Others are not. I have written this guide to be useful regardless of whether you're near northeast Florida — the framework and questions apply anywhere you're considering peptide therapy.

If you're in the Ponte Vedra / Jacksonville area

You can book a consultation at Youthful Medical Spa to discuss whether peptide therapy is appropriate for you. All prescriptions are made by a licensed clinician, and compounded medications come from licensed pharmacies with batch-specific testing.

youthfulmedicalspa.com

If you're elsewhere

Use the framework in Section 6 and the questions in Section 7 to evaluate providers near you. A good provider anywhere in the country will welcome the questions you've learned to ask.

Final note

I wrote this document because when I started studying peptides seriously, I couldn't find a resource that treated the reader like an adult. Everything was either sales copy pretending to be education, or dry regulatory documents that took hours to parse.

The peptide space is genuinely one of the most interesting frontiers in clinical medicine right now. It's also one of the most confusing, and the confusion is being exploited on all sides — by influencers selling protocols, by clinics selling packages, by gray-market vendors selling contaminated vials. My hope is that after reading this, you have a lens you can use to evaluate what you see, ask better questions, and — if you decide peptide therapy is right for you — work with a provider who actually deserves your trust.

This document will change. I'll publish version 1.1 after the July 23-24 PCAC meeting reveals what the FDA advisory committee actually recommends. If you want the update when it's ready, follow me on Instagram at @nurse_practitioner_berkleigh — the updated PDF will go out to my email list first.

— Berkleigh

Follow the work

Instagram: @nurse_practitioner_berkleigh

Links & resources: linktr.ee/berkleighmahan

Disclaimer, repeated because it matters

This document is educational and reflects my understanding as a graduate nursing student, not the practice of medicine. Nothing here is a substitute for a real clinical relationship. Do not use this document to self-treat or to select peptides for personal use without a licensed prescriber. Regulatory status changes; verify current FDA guidance before making any decision that depends on it.